



ST. JOHN'S MEDICAL COLLEGE HOSPITAL
Bengaluru 560034, Tel +91 80 22065000
DEPARTMENT OF NUCLEAR MEDICINE



Patient ID	5271419	Patient Name	KRISHNAIAH SHETTY
Gender	Male	Study Date	23-05-2025 10:36:46
Age	71Y	Reported Date	23-05-2025 13:41:43
Study Type	PT	Ref Physician	DR DODDI NEEHARIKA

No significant pelvic / inguinal lymphadenopathy.

Musculoskeletal system: Degenerative changes are noted in the spine. D7 vertebral haemangioma noted. No definite focal osteolytic/sclerotic or FDG avid skeletal lesion seen.

Vascular Structures:

Atheromatous changes are seen in aorta. Major vessels in the neck, chest, and abdomen are otherwise unremarkable.

IMPRESSION:

In this known case of multiple ICSOL ; the PETCT study shows:

- Mild metabolically active multiple heterogeneously enhancing SOL seen in brain, as described above - ? infective etiology.
- Metastasis from occult primary is another possible differential, in view of no definite metabolically active lesion in rest of the body; Multicentric glioma appears less likely in view of well defined nature of the lesions.
- Heterogeneous increased metabolic activity in stomach predominantly along the greater curvature, with focal increased metabolic activity at pyloric antrum, with no corresponding structural lesions on CT - ? inflammatory (suggested upper GI endoscopy correlation, if any clinical discrepancy).
- Other findings as described above.


Dr. S. Chandra Teja Reddy
Assistant Professor
Nuclear Medicine (KMC - 170092)

Dr. Roger Anthony Manuel
Radiologist
KMC-90884



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Parotid and submandibular glands are unremarkable.
Pharyngeal mucosal space, parapharyngeal, retropharyngeal, carotid spaces are unremarkable.
Larynx and cervical trachea are unremarkable.
Thyroid gland appears unremarkable. No significant enlarged or FDG avid cervical lymph nodes.

Chest:

Non-FDG avid ill-defined opacity seen involving subpleural region of superior lingular segment of left upper lobe measuring up to 4 mm. Few small thin walled air cysts noted in left lung. Otherwise, lungs are clear.

No evidence of pleural effusion or pneumothorax.

Few minimal FDG avid upper and lower paratracheal, para-aortic lymph nodes noted; right lower paratracheal node showing central necrosis measuring up to 9 x 7 mm with SUVmax 2.0.

Oesophagus appears unremarkable. Cardia is unremarkable. No pericardial effusion.

Chest wall and axillae are unremarkable.

Abdomen and pelvis:

Liver: Measures 11.6 cm craniocaudally. No focal FDG avid lesion. No IHBR dilatation. Portal vein, hepatic veins and intrahepatic IVC are normal.

Gall bladder: *few small calculi seen at the neck of gallbladder with pericholecystic stranding.* CBD is not dilated.

Spleen: Unremarkable. Pancreas: Unremarkable. Adrenal glands: Unremarkable.

Kidneys: Left prominent extrarenal pelvis noted. Otherwise, kidneys appear unremarkable.

G.I tract: *Stomach is moderately distended with diffuse heterogeneous FDG uptake, predominantly along the mucosa of greater curvature with prominent rugal folds (SUVmax 4.7). Focal area of mild increased FDG uptake in pyloric region with no obvious structural lesions on CT (SUVmax 5.2) - ? inflammatory.* Heterogeneous FDG uptake is seen in few bowel segments, likely non-specific.

Rest of the small and large bowel segments are otherwise unremarkable.

Peritoneum and retroperitoneum: No evidence of ascites or pneumoperitoneum.

No significant mesenteric nodes. No significant retroperitoneal lymphadenopathy.

Pelvis: Mild prostatomegaly with no focal FDG avid lesions. Bilateral seminal vesicles are unremarkable.

Urinary bladder is partially distended.



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¹⁸F-FDG PET-CT SCAN

Technique: Whole body PET followed by multislice CT images was acquired in 3-D mode from vertex to mid-thigh ~ 56 min after IV injection of 6.9 mCi of ¹⁸F-FDG followed by dedicated PET/CT brain using *Biograph Horizon PET/CT scanner*. Iodinated IV contrast media 68 ml was used for enhancement. Iterative reconstruction of the acquired data was done to obtain fused PET-CT images in transaxial, coronal, and sagittal views. SUV values were calculated based on body weight.

Patient parameters: FBS: 109 mg/dl. Serum Creatinine: 0.7 mg/dl.

Indication: Multiple intracranial SOL.

Reference made to MRI brain dated 20/5/2025

Findings:

- Mild FDG avid heterogeneously enhancing well defined lesion noted in the right parietal region with thick irregular wall and central necrosis measuring 3.3 x 2.0 x 3.1 cm (AP x TR x CC) with SUVmax 7.0. Significant perilesional edema noted with no obvious midline shift. Multiple other similar lesions with similar characteristics noted in the left parietal lobe, right temporal lobe, bilateral occipital lobes, adjacent to frontal horn of left lateral ventricle & left thalamus, largest measuring 2.2x2.0cm in left parietal lobe with SUVmax: 6.1. Effacement of cortical sulcations adjacent to the lesions due to edema noted.
- No abnormal FDG avid lesion in the lungs, liver, adrenals, or skeleton.

Other findings:

Physiological FDG concentration is seen in the brain, salivary glands, oral cavity, heart, GIT, kidneys, and urinary bladder.

Head & Neck:

Brain lesions as described above. Prominent cisterna magna noted. Paranasal sinuses are unremarkable.